

Housebound patients ~general principles and practical implications~

1) It is recognised that in most circumstances it is advantageous for patients to be seen for their health needs in the surgery setting, (more time efficient, better access to (diagnostic) equipment, availability of other professionals)

2) However, there are circumstances either temporary (i.e. following surgery) or permanently that it is not possible for the patient to attend surgery and are classified as “housebound”

3) The following definition can be used; **Housebound patients are those who are unable to leave their home environment due to physical or psychological illness – severe agoraphobia and in keeping with ability to leave for any reason, or because doing so would require significant and taxing effort that would harm their mental health.**

4) In the context of GP home visits, **housebound** usually means:

- The person cannot leave their home safely on their own due to illness, disability, or frailty.
- Leaving the home requires substantial effort, support, or planning (e.g., special transport, accompaniment by more than one carer).
- The person’s medical or functional condition makes it inappropriate or unsafe to expect them to attend a GP surgery.
- This is not just about convenience — being housebound means that attending an appointment outside the home would cause undue distress, risk, or deterioration in health.

5) In the above circumstances it is appropriate for the patient to be seen at home if face to face contact with a health professionals is needed.

6) It’s worth noting:

- Being elderly does not automatically mean being housebound.
- Patients may be temporarily housebound (e.g., after major surgery or acute illness) or permanently housebound.
- “Housebound” does not usually include people who can leave home with relative ease (e.g., with a stick, frame, or taxi service).
- Lack of transport is not a valid reason to be considered housebound.
- There are not many circumstances that an ill child cannot be brought to the surgery.

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Some practical implications

- 1) It should be clear from the medical records if a patient is permanently housebound (if only so we do not need to ask the question when a visit is requested.) There are read-codes for “housebound” and “temporarily housebound” which should be used.
- 2) It should be clear on our website what our definition and policy is with regards to housebound patients and the above text can be used for this purpose.
- 3) When a patient asks for a home visit, the care coordinator/receptionist taking the request needs to check (emis problem list) whether the patient has already been classified as permanently housebound or otherwise have regards to the following;

Consider a patient potentially housebound if:

- They cannot leave their home safely without the help of (more than 1?) other person. (maybe delete)
- Leaving home requires considerable physical effort, planning, or support (e.g. specialist transport, multiple carers).(hospital transport only??)
- Attending the surgery would cause significant distress, risk, or deterioration in health.
- They are temporarily unable to travel due to acute illness, recovery from surgery, or a disabling condition.

Do not consider housebound if:

- The patient is able to leave home independently or with minimal support (e.g. by taxi or with one carer).
- The main issue is lack of transport or convenience.
- The patient declines to attend surgery despite being able to do so.

If unsure:

- Discuss with the GP or duty doctor for case-by-case assessment.
- Remember that housebound status may be **temporary or permanent**.

It is worth noting that even if a patient is housebound, a GP might still consider a visit unnecessary, i.e. a phone call or a referral to another health professional could be an option; so never promise or confirm a visit will take place but state *“I will put your request to one of the GPs. They might call you to assess your request. What is the best person/ phone number to use if further information is needed”*

Finally; Regardless of definitions and questions there will always be circumstances that we cannot be sure whether a patient needs a visit or not. You might apply the following general principle here; if in doubt; visit (and if you feel like it; argue/ discuss afterwards).